



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : رضا نبيل على ابراهيم

العنوان : --

الحي / المدينة : عين شمس

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 27004160100068 رقم التليفون : 01120487116

العمر (تاريخ الميلاد) : 16-04-1970 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رضا نبيل على ابراهيم National ID: 27004160100068	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية الكويت
بطاقة تعقب الهوية

رضا
نبيل علي ابراهيم
٢٠٠٠
باب الشعيرة - القاهرة

٢٧٠٠٤١٦٠١٠٠٠٦٨

١٩٧٠/١٠

KW8953050

٢٠٢٤/٢٠٠٠٤١٦٠١٠٠٠٦٨

حاصله على دبلوم تجارة

متروجة
مستلة
رمضان عثمان محمود عثمان
٢٠٣٨/١١/٣٠

البطاقة سارية حتى

Name: Mrs. reda nabil ali
6/12/2025

Left Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: right MRM

Findings:

Left breast

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- An enlarged level I axillary lymph node measuring 10 x 7.5, with eccentric hilum as well as increased cortical thickness reaching to 5mm for close follow up

Conclusion:-

- Left breast (BIRADS 1): normal study.
- Clear right mastectomy bed
- Indeterminate right axillary lymph node for close follow up

Dr farah elshobaky

Name: رضا نبيل علي MR. : 3059	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه	
The patient was initially staged as cT4d N2 M0 invasive ductal carcinoma, ER-negative, PR-negative, HER2-positive. Neoadjuvant chemotherapy consisted of 4 cycles of AC, followed by a left modified radical mastectomy on 15/08/24 and subsequent radiotherapy. The patient then received phesgo for 12 cycles, the last dose administered in 12/2024. Surveillance mammography in 04/2025 demonstrated stable right-breast duct ectasia (BIRADS 3). Ultrasound performed in 07/2025 upgraded the finding to BIRADS 4, and a biopsy of an old collagenous fibroid was obtained. The patient has completed 12 cycles of phesgo and 4 cycles of AC. The patient has a history of right-breast cancer diagnosed in 2018, treated with right modified radical mastectomy (stage not specified) and classified as Luminal A. Adjuvant therapy included 3 cycles of EC and 3 cycles of TXT, followed by radiotherapy and aromatase inhibitor therapy for 5 years, completed in 02/2024. PET-CT in 2024 revealed mediastinal lymph nodes consistent with old granuloma, which were not biopsied and are being followed. Surveillance mammography in 12/2025 showed a clear right mastectomy bed but identified an indeterminate right axillary level 1 lymph node measuring 10 × 7.5 mm with eccentric hilum and increased cortical thickness of 5 mm, warranting close follow-up. Bilateral breast ultrasound in 08/2026 was negative. A new contrast-enhanced CT of the chest, abdomen, and pelvis with comparison has been requested. The patient has completed 3 cycles of EC and 3 cycles of TXT.	
تحريراً في ,	
التاريخ :	التاريخ : 19-09-2026 الوقت :
الطبيب المعالج :	حنان عزت شفيق تاودروس
يعتمد ,	



MDT form

A-patient ID: رضا نيل علي بن احمد

Sex: Female

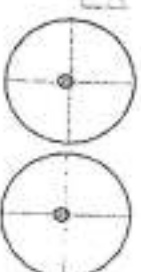
3- Medical number: 83191766

Old Med I

Old Med I 713-2024

2-ID number: 27004160100068

Patient Age: 54 Y



Family history	Family history	Phone	01120467116 01226880861	BM1	BM1	REASON OF MDT	REASON OF MDT
Marital status	Marital status	NRS Score	0/5 Score	GROUP	GROUP	Date of MDT	Date of MDT
Co-morbidity	Co-morbidity	Macropausal status	Macropausal status	Initial STAGE	Final STAGE	30/04/2024	30/04/2024

Breast Imaging :

Clinical data

Mets Workup :

PET CT 25/2/2024 MEDIASTINAL LN

BX IDC	ER 5/8	PR 0/8	Her 2 Neu 0	SISH	Ki67 10%
ER Comment:	PR Comment:	HER 2 Neu Comment:	SISH Comment:		

T	N	M
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Sonomammography

PATIENT HISTORY

9/2018 R.T BREAST CA ---> R.T MRM TXNXMO
ADJ EC X3 ---> TXT X3 ---> PORTH ---> AI X5 2/2024
PET CT 2024 ---> MEDIASTINAL L.N --- BX WAS PLANNED BUT CANCELLED
FOR BRONCHOSCOPIC BIOPSY FROM MED LNS AS SHE HAS PULMONARY FIBROTIC CHANGES AND GGO
NEW TUMOR MARKERS CA15.3, CEA: normal

MDT OUTCOME & PLAN

stay on follow up ,, repeat PET or CT with contrast after 2 months and reconsult

MEDICAL ONCOLOGY

SIGNATURE

MEDICAL ONCOLOGY

RADIOLOGY

PATHOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

جمهورية مصر العربية
وزارة الداخلية
محافظة القاهرة - القاهرة

رضا

نبيل طلي إبراهيم

٢٠ ش. بير المطور - ش. الصين

باب النورية - القاهرة



KM8953050

٢٧٠٠٤١٦٠١٠٠٠٦٨

Patient Name :
Patient No. : 27004160100068
Path No. : SR-369/24
Sex : Female
Age : 53 Y
Referring Doc :

Request No. : 13005402
Request Date : 19/03/2024
Verification Date : 31/03/2024
Financial Category :
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical data :
Breast mass.

Nature of specimen :
Slide revision.

Gross examination:
6 blocks (14/5/8) + 9 slides.

- Microscopic examination :
- Histological type: Invasive lobular carcinoma.
 - Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
 - DCIS: Not detected.
 - Angiolinvasion: Not detected.
 - Skin Involvement : Free nipple and areola
 - Lymph node involvement:Free received nodes (0/7).
 - Received margins: Free.
 - PT:
 - ER: Strong nuclear reaction in 70% of tumor cells (score 8/9).
 - PR: Strong nuclear reaction in 10% of tumor cells (score 3/8).
 - Her2: No membrane reaction in tumor cells (score 0).
 - Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Breast mass . Slide revision:
Invasive lobular carcinoma, Grade II .
Free nipple and areola.

Verified by :
dr.esraa magdy

Printing Date : 03/04/2024 09:30:37

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name	: محمد علي احمد
Patient No.	: 27004160100068
Path No.	: SR-369/24
Sex	: Female
Age	: 53 y
Referring Doc :	

Request No.	: 13005402
Request Date	: 19/03/2024
Verification Date	: 31/03/2024
Financial Category	: ٤٢٣ - ٤٢٣
Department	: Out Patient
Registered by	: marwa mohamed youss

Pathology Report

- Free received margin.
- Free received lymph nodes: (0/7)
- IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Prof. Gamal Abdelatty
Dr. Sagda Elkady

Verified by :
dr.esraa magdy

Printing Date : 03/04/2024 09:20:37

Page 2 of 2

Printing By : marwa mohamed 20235



Room No. (4) - First Floor-Building No. 937, Site C, Yerniche - Miter El Qadeem - Cairo, Egypt

Patient Name: Reda Nabil Ali Ibrahim	Date : 07/10/2024
Exam : C.T ABDOMEN & PELVIS WITH CONTRAST	Age : 54 Years (s)
Medical ID : 713/2024	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Status post right MRM, compared to last PET CT study (attached CD) dated 25/2/2024 done outside.

I Chest:-

- Evident right mastectomy with clear operative bed, no gross tumoral tissue recurrence or residual.
- Stationary right axillary level 1 few prominent lymph nodes are seen the largest still measures about 1.2x0.9 cm.
- Still noted multiple enlarged rather amalgamated both hilar, different group mediastinal lymph nodes with the largest seen at the subcarinal region measures collectively 2.5 x2.5 cm, some of them show multiple calcific foci.
- Stationary few lower cervical enlarged lymphnodes with the largest on the right side measures 1.5 x0.5 cm.
- Rather stationary previous multiple bilateral multisegmental sub/centimetric scattered pulmonary nodules, the largest is still seen at the left upper lung apicoposterior segment measuring about 8 mm.
- Bilateral upper and middle lobar central and peripheral subpleural area of reticulonodular infiltrate as well as thick atelectatic bands with traction bronchiectasis.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged liver with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries

Patient Name: Reda Nabil Ali Ibrahim	Date: 07/10/2024
Exam: C T ABDOMEN & PELVIS WITH CONTRAST	Age: 54 Years
Medical ID: 713/2024	

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Bulky right adrenal gland show rather well-defined rounded fatty attenuation lesion, adenoma.
- Normal appearance of the pancreas, aorta, I.V.C.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with right adnexal lesion closely related to the sigmoid with minimal pelvic fluid, for pelvic ultrasound correlation.
- Bilateral few prominent external iliac lymph nodes.
- Scanned skeleton:
 - Still noted L1, L3 and L4 vertebral bodies lytic bone lesions.
 - Still noted D11 vertebral body lytic bone lesion with vertical striations likely osseous hemangioma.
 - Spondylodegenerative changes.

Conclusion:

Status right MRM with rather stationary course since last study.

Dr. Karam Mohamed, FEBR, MD



Patient's Name: Reda Nabil Ali Ibrahim
Date: 2026-09-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION

REVEALED:

I- CHEST:

- Bilateral diffuse interlobular and intralobular septal thickening resulting in reticular pattern with basal and peripheral predominance.
- The condition is associated with traction bronchiectasis, subpleural fibrotic bands and honeycombing.
- Multiple mediastinal and bilateral enlarged hilar lymph nodes with calcific foci the largest measures 32 x 24 mm.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Multiple hepatic hypodense focal lesions the largest is seen in the left lobe measuring 54 x 53 mm showed heterogeneous mainly peripheral enhancement
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C. T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple vertebral bodies and right iliac bones lytic lesions

Opinion:

- ❖ Picture is suggestive of interstitial lung disease ? Idiopathic pulmonary fibrosis (IPF) / usual interstitial pneumonia (UIP).

- ❖ Multiple osseous metastatic lesions.

Dr. Asser Ahmed